

KING FAHD CENTRAL HOSPITAL (KFCH)

Comprehensive Stroke Program

IV THROMBOLYSIS ELIGIBILITY & ADMINISTRATION CHECKLIST

Document ID	SU-003-FORM-01	Version	1.0
Effective Date	_____	Review Date	_____

Patient Name / MRN	_____	Date / Time	_____
DOB / Age	_____	Weight (kg)	_____
LKW / Symptom Onset	_____	Baseline NIHSS	_____

A. Immediate Stroke Assessment

Assessment Item	Yes / No / N/A	Comments / Time
Clinical diagnosis of acute ischemic stroke with a measurable neurological deficit	☐ Yes ☐ No ☐ N/A	
Last Known Well / symptom onset documented	☐ Yes ☐ No ☐ N/A	
NIHSS completed and documented	☐ Yes ☐ No ☐ N/A	
Point-of-care blood glucose obtained; hypoglycemia corrected if present	☐ Yes ☐ No ☐ N/A	
Non-contrast CT/MRI excludes intracranial hemorrhage	☐ Yes ☐ No ☐ N/A	
Blood pressure assessed and managed to IV thrombolysis treatment threshold	☐ Yes ☐ No ☐ N/A	
Medication history reviewed, including anticoagulants and recent antithrombotic use	☐ Yes ☐ No ☐ N/A	

B. Treatment-Window / Imaging Eligibility

Assessment Item	Yes / No / N/A	Comments / Time
Patient is within an approved IV thrombolysis treatment window under the current institutional protocol	☐ Yes ☐ No ☐ N/A	
For wake-up/unknown-onset or extended-window treatment, required advanced imaging criteria are satisfied when applicable	☐ Yes ☐ No ☐ N/A	
No imaging finding or clinical circumstance makes IV thrombolysis inappropriate under the approved protocol	☐ Yes ☐ No ☐ N/A	

Use the current KFCH IV thrombolysis protocol and current approved stroke guideline criteria. Do not delay treatment for tests that are not clinically required.

C. Contraindication / High-Risk Screen

Assessment Item	Yes / No / N/A	Comments / Time
No intracranial hemorrhage on initial imaging	☐ Yes ☐ No ☐ N/A	
No active internal bleeding or other absolute exclusion under the approved institutional protocol	☐ Yes ☐ No ☐ N/A	
Recent intracranial/intraspinal surgery, significant head trauma, or prior intracranial hemorrhage reviewed	☐ Yes ☐ No ☐ N/A	
Recent major surgery/trauma and recent gastrointestinal/genitourinary bleeding reviewed	☐ Yes ☐ No ☐ N/A	
Platelet count/coagulation status reviewed when clinically indicated or anticoagulant exposure is suspected/known	☐ Yes ☐ No ☐ N/A	
Recent DOAC, warfarin, heparin, or other anticoagulant exposure assessed according to protocol	☐ Yes ☐ No ☐ N/A	
Blood pressure can be maintained within treatment threshold	☐ Yes ☐ No ☐ N/A	
Potential stroke mimic / seizure at onset / pregnancy-postpartum / other special circumstance assessed when applicable	☐ Yes ☐ No ☐ N/A	



D. Pre-Administration Safety Verification

Assessment Item	Yes / No / N/A	Comments / Time
Actual measured body weight documented	☐ Yes ☐ No ☐ N/A	
Thrombolytic agent selected according to current KFCH approved formulary/protocol	☐ Yes ☐ No ☐ N/A	
Dose independently calculated and verified by two qualified clinicians according to institutional medication-safety policy	☐ Yes ☐ No ☐ N/A	
Drug, concentration, dose, route, and administration method verified	☐ Yes ☐ No ☐ N/A	
Baseline vital signs and neurological status documented	☐ Yes ☐ No ☐ N/A	
IV access secured and required blood samples obtained without unnecessary treatment delay	☐ Yes ☐ No ☐ N/A	
Treating physician confirms eligibility and documents treatment decision	☐ Yes ☐ No ☐ N/A	
Patient/family informed of expected benefit and major risks according to KFCH consent policy; emergency exception documented when applicable	☐ Yes ☐ No ☐ N/A	

E. Thrombolytic Administration Record

Agent	☐ Alteplase ☐ Tenecteplase ☐ Other: _____	Patient weight	_____ kg
Calculated dose	_____	Verified dose	_____
Administration start time	_____	Door-to-needle time	_____ min
Administered by	_____	Independent verifier	_____

F. Post-Thrombolysis Monitoring & Precautions

Assessment Item	Yes / No / N/A	Comments / Time
Neurological status and vital signs monitored at protocol-defined frequency	☐ Yes ☐ No ☐ N/A	
Blood pressure maintained within post-thrombolysis target specified by the approved KFCH protocol	☐ Yes ☐ No ☐ N/A	
No antiplatelet or anticoagulant therapy given during the first 24 hours unless specifically indicated and authorized	☐ Yes ☐ No ☐ N/A	
Invasive procedures / arterial or venous punctures minimized during the early post-treatment period	☐ Yes ☐ No ☐ N/A	
Follow-up CT/MRI obtained at approximately 24 hours, or earlier for neurological deterioration	☐ Yes ☐ No ☐ N/A	
Any sudden neurological deterioration, severe headache, acute hypertension, nausea/vomiting, or bleeding triggers immediate hemorrhage-response pathway	☐ Yes ☐ No ☐ N/A	
Post-treatment disposition to an appropriately monitored Stroke Unit/ICU setting documented	☐ Yes ☐ No ☐ N/A	

G. Final Treatment Decision

Decision	☐ IV thrombolysis administered ☐ Not administered
If not administered, reason	_____
Variance / delay reason (if applicable)	_____

Treating Physician	_____	Date/Time	_____
Stroke Nurse / RN	_____	Date/Time	_____

Document linkage: Supporting form to the KFCH Stroke Unit Policy & Procedure Manual 2026 and the approved KFCH IV Alteplase (tPA) Preparation & Administration Protocol. This checklist supports, but does not replace, clinical judgment or the current approved protocol.